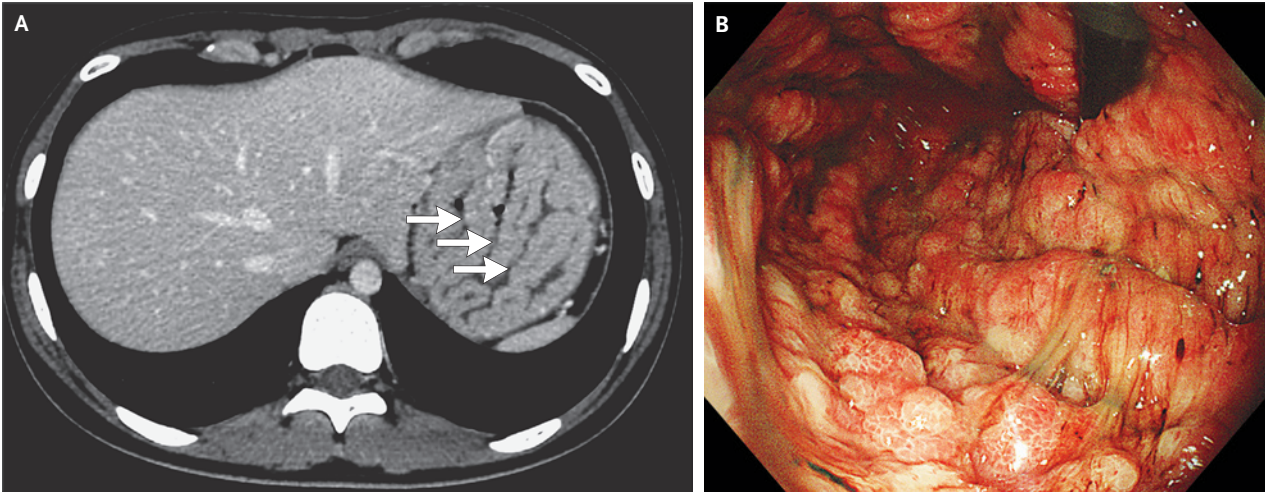


IMAGES IN CLINICAL MEDICINE

Ménétrier's Disease



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A PREVIOUSLY HEALTHY 16-YEAR-OLD GIRL WAS ADMITTED TO THE HOSPITAL with a 2-month history of leg swelling. She had no gastrointestinal symptoms. On physical examination, pitting edema of both legs and mild periorbital edema were noted. Laboratory testing showed a serum albumin level of 26 g per liter (normal range, 38 to 54); a urinalysis and the results of tests of liver and kidney function were normal. To look for gastrointestinal sources of protein loss, computed tomography of the abdomen was performed and revealed diffuse thickening of the gastric wall with enlarged gastric folds (Panel A, arrows). On gastroscopy, markedly edematous gastric rugae were seen, along with nodularity, mucosal erosions, and overlying yellow mucus (Panel B). A biopsy of the gastric mucosa revealed foveolar hyperplasia with tortuous and dilated glands. A diagnosis of Ménétrier's disease — also known as hypertrophic gastropathy — was made. Ménétrier's disease is an idiopathic hyperproliferative disorder of the gastric foveolar epithelium in which hypersecretion of mucus results in malabsorption and protein-losing gastropathy. Patients typically present with gastrointestinal symptoms, peripheral edema, and hypoalbuminemia. Treatment with a high-protein diet was started, as were medications to eradicate a concurrent infection with *Helicobacter pylori*. At a 3-month follow-up visit, the patient's edema and hypoalbuminemia had resolved.

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